CareModelOps | Investment Decision

Executive decision memorandum • Independent strategic case study • 29 October 2026

Decision requested

Authorize a bounded 90-day discovery, paired evaluation and regulated-deployment assessment for clinical-workflow independent software vendors (ISVs), with oncology as the first specialty workflow. Do not authorize broad product launch, reseller commitments, production PHI processing or large incumbent integrations. At Day 90, decide GO / ITERATE / STOP based on externally observed buyer demand, matched task-level model performance, workflow unit economics and deployment gates.

Why this investment thesis merits testing

Established evidence	What it tells us	What it does NOT establish
M6.1: market selection	Clinical workflow ISVs led a weighted *analyst* market score; oncology is a beachhead inside that segment.	Verified segment TAM, demand, contract value or partner conversion.
M6.2: consumption economics	Base: ~$125.4K first-year modeled API billings; ~$268.8K annualized Month-12 billings. Annualized $1M requires ~186 base-equivalent paying ISVs.	Anthropic recognized revenue, internal gross margin, CAC, validated usage or actual customer willingness to pay.
M6.3–M6.4: adoption and buyer evidence	A defined developer funnel, five P0 capabilities, 20 public-company profiles and six discovery candidates.	External interviews, signed design partners, conversion rates or enterprise authorization.
M6.5: competitive and partner position	Native source citations and multi-route distribution provide credible product hypotheses; direct API, cloud procurement and implementation partners serve different roles.	Superiority over GPT/Gemini on current oncology workflows or confirmed co-selling agreements.
ChartGround M1–M5	Locally demonstrated Claude calls, tenant isolation, token metering, partner attribution, simulated billing, spend controls, dashboards; 151 offline tests passed.	Production-scale availability, clinical validation, HIPAA approval or realized commercial revenue.

Financial underwriting — distinguish downside from prediction

M6.2 scenario	Year-1 API billings, modeled	Month-12 scale
Conservative	$1,524	10 ISVs; $3,223 annualized API billings
Base	$125,412	50 ISVs; $268,849 annualized API billings
Expansion	$2,384,434	200 ISVs; $5,065,100 annualized API billings

M6.6 adds two assumption-driven stress views: (1) assigning illustrative weights of 60% conservative / 35% base / 5% expansion yields ~$164,030 in scenario-weighted first-year API billings; these are not empirically estimated outcome probabilities. (2) applying 75% workflow realization and 90% incremental price realization to the M6.2 base produces ~$84,653 of first-year billings. These are alternative lenses, not independently validated forecasts. Neither represents contribution profit: Anthropic inference costs, sales/servicing costs and accounting terms are unknown.

90-day evidence plan and decision boundaries

Period	Work / accountable lead	What must be true
Days 0–30	Product + developer relations: 6–8 substantive external ISV / buyer interviews from six publicly researched targets plus adjacent firms.	At least two recurring, expensive problems externally corroborated; real decision-maker and technical sponsor identified.
Days 31–60	Evaluation + clinical reviewers: pre-registered blinded matched cases across current Claude, GPT, Gemini and no-AI baseline.	No unacceptable high-severity clinical error; acceptable evidence-grounding, review time, latency and total cost per accepted case.
Days 31–75	Solutions engineering + security/legal: compare first-party API, Bedrock, selected Microsoft and Google routes.	Chosen path has identified operator, eligible features, agreement/BAA, prompt/log/metadata flow and approvals for planned use.
Days 61–90	Product + finance + partnerships: refresh ISV consumption, margin/ROI, procurement route and implementation economics from evidence.	A credible go/iterate/stop decision with actual workflow assumptions, partner roles and risk owner.
Days 90–180	Only if Day-90 gates pass: controlled pilot, measure paid non-test production use and mature cohort retention.	Real external workload and paid consumption. No counting demo tokens or synthetic invoices as production adoption.

Partner sequencing and model-selection rule

Begin with direct first-party API developer discovery, subject to an approved synthetic/nonclinical environment. Use Amazon Bedrock, Microsoft Foundry or Google Cloud pathways only where an actual buyer requires the corresponding procurement and data-processing architecture; verify service eligibility and Business Associate Agreement coverage individually. Consult qualified implementation partners (including firms in Anthropic's announced partner ecosystem) if demand is validated**. Evaluate Flatiron, RaySearch, Elekta and Varian only as possible later oncology workflow integration or distribution channels**.

Choose the model based on blinded accepted-workflow quality and economics—not a general "best clinical model" claim. Claude native document citations and strict structured JSON output cannot be combined in one API request under the documented constraint; any two-stage workaround must be measured for added latency, cost and error risk.

Risk-adjusted decision logic

GO (authorize controlled next phase)	ITERATE (narrow the wedge)	STOP / change market
Buyer and developer sponsor identified; matched evaluation meets prespecified safety/quality thresholds; production volume and unit economics are supportable; deployment architecture is approvable.	Developer interest but unclear willingness to pay, borderline clinician review economics, or solvable onboarding/technical frictions.	No recurring workflow with measurable value, unacceptable error rate, inability to meet PHI requirements, or structurally uneconomic usage.

Resource request — conditional, not an Anthropic budget estimate

Proposed exploratory capacity: approximately 0.5 product manager, 0.5 developer-relations resource, 0.25 solutions/evaluation engineer and 0.25 privacy/security advisor for three months (4.5 full-time-equivalent months). The companion workbook includes purely illustrative fully loaded rates and a modeled ~$136K capacity envelope.

Executive questions this case should withstand

Executive challenge	Response / action
Why oncology first if clinical workflow ISVs rank higher?	Oncology is the initial workflow specialization within the broader clinical-workflow ISV category, not a separate mutually exclusive segment.
Why is $1M annualized usage hard?	M6.2 modeled low API cost per modest-token task; revenue requires significant recurring workloads, more downstream orgs and potentially higher token intensity.
Is this differentiated from existing ambient AI?	No claim of generic ambient-documentation advantage. Evaluate longitudinal evidence synthesis, contradiction handling, citations and clinician-review workload.
Why cloud marketplaces instead of the Anthropic API?	They are procurement/deployment alternatives, not automatically separate incremental revenue channels. Verify billing chain and HIPAA eligibility.
Has Anthropic already acquired these customers?	No. Six discovery candidates are publicly researched hypotheses; none is verified to be an Anthropic buyer or committed partner.
Are we asking for production approval?	No. Day-90 decision requests evidence-based authorization for a controlled next phase only. PHI processing requires separate legal/security/clinical gates.